



Achieving the Triple Aim:

*The Simultaneous Pursuit of
Population Health,
Enhanced Individual Care, and
Controlled Costs for a Population*

Aims for Session

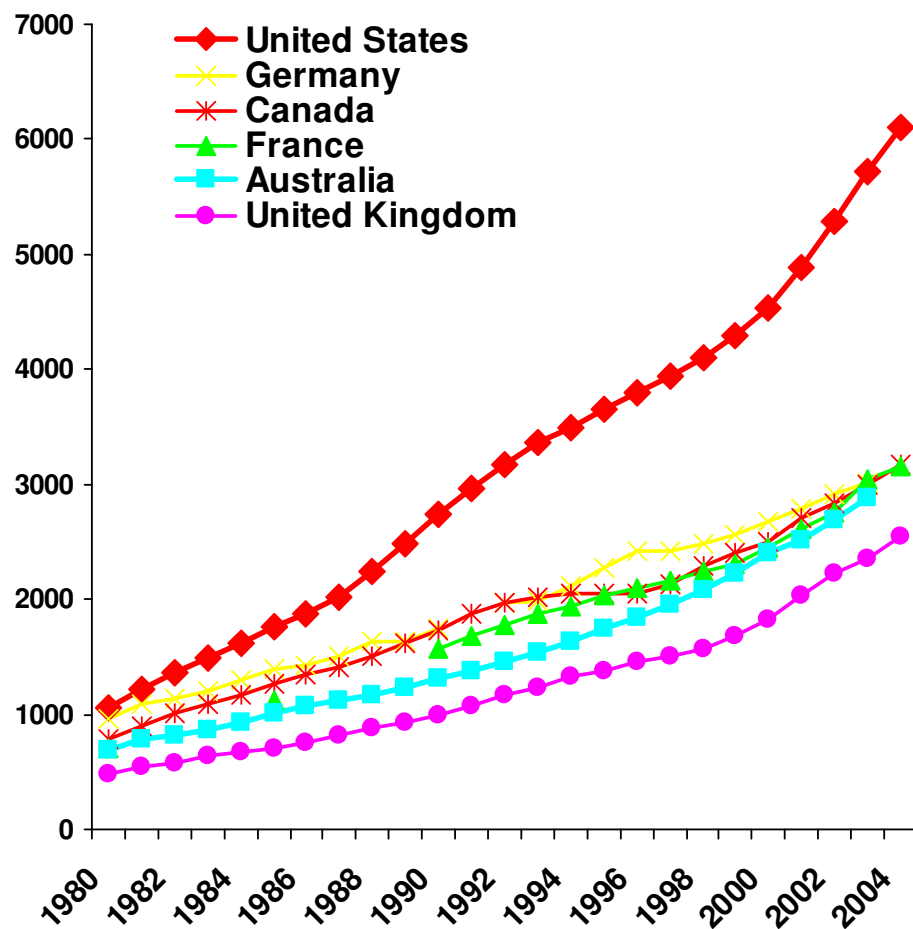
- Describe dynamics giving rise to Triple Aim framework
- Share current site work illustrating results in the three aims
- Describe frontiers in pursuing the Triple Aim and emerging links to other efforts



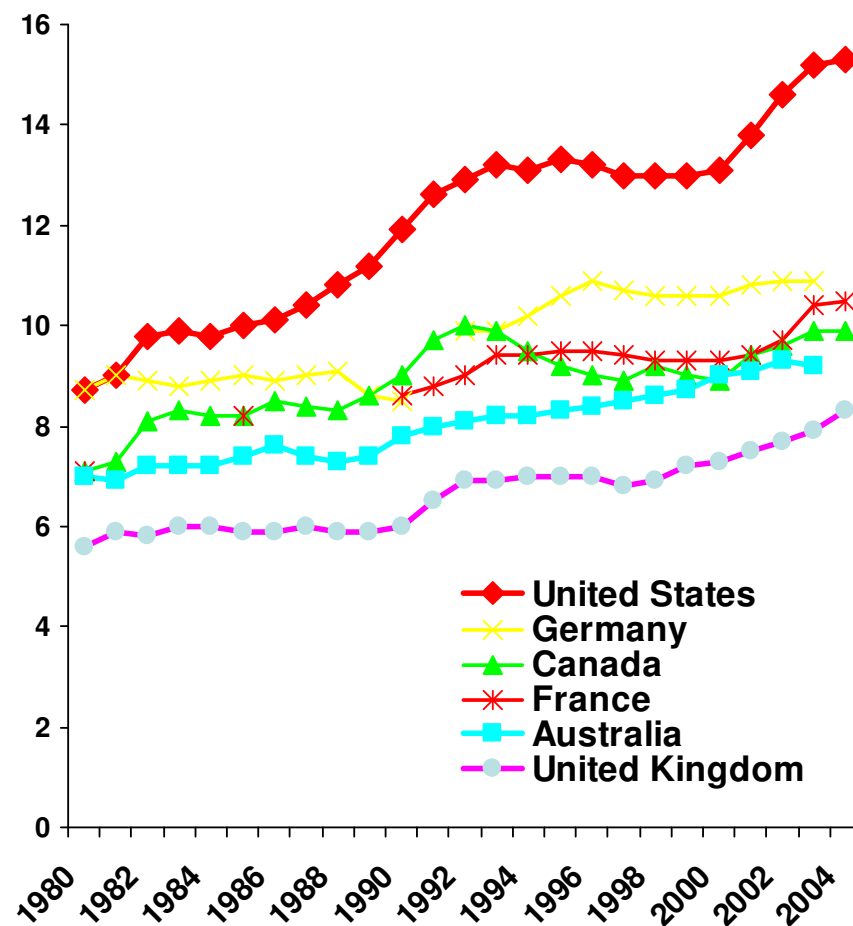
What Are the Dynamics Underlying the Problem?

International Comparison of Spending on Health, 1980–2004

**Average spending on health
per capita (\$US PPP)**

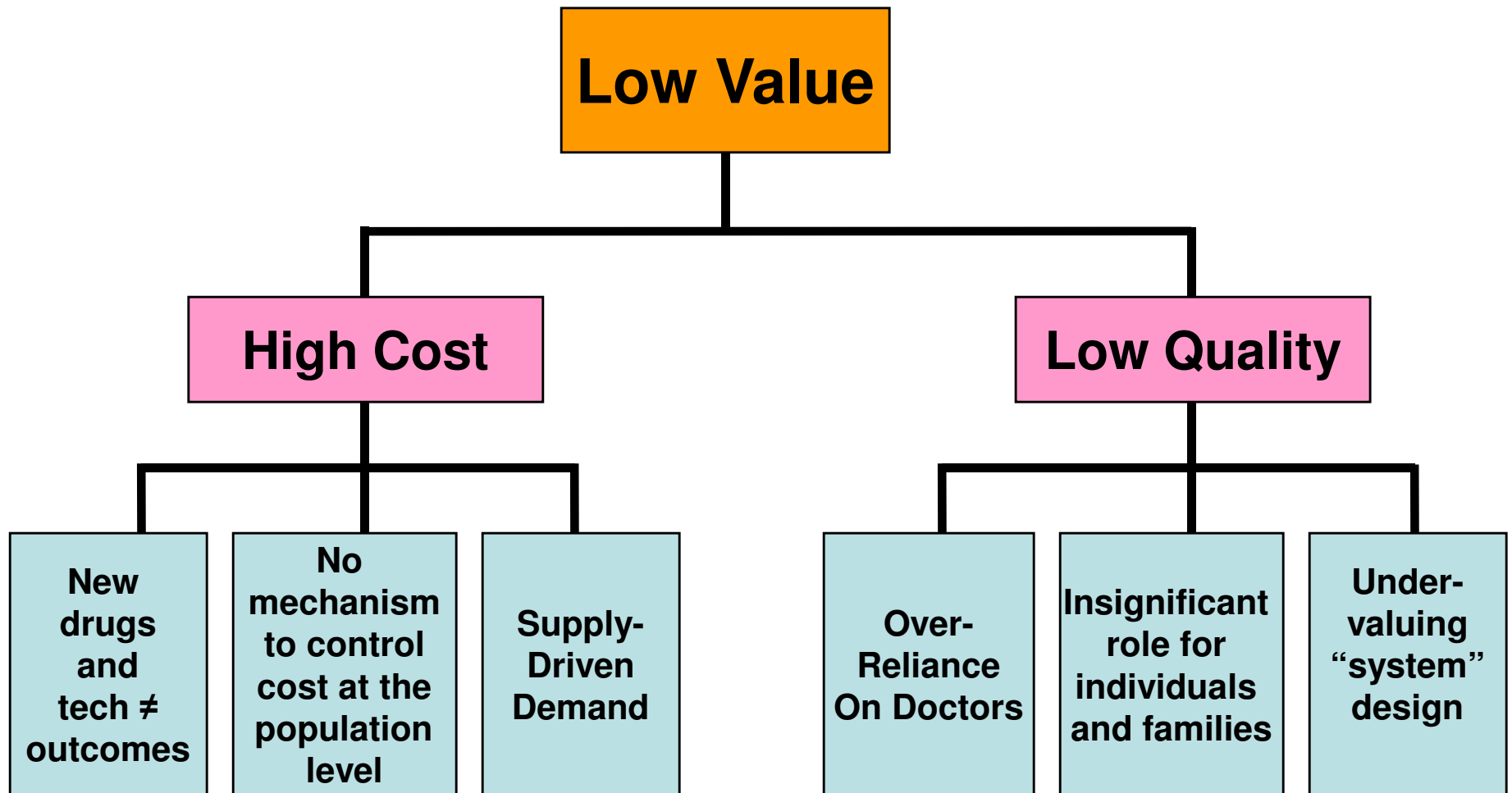


**Total expenditures on health
as percent of GDP**



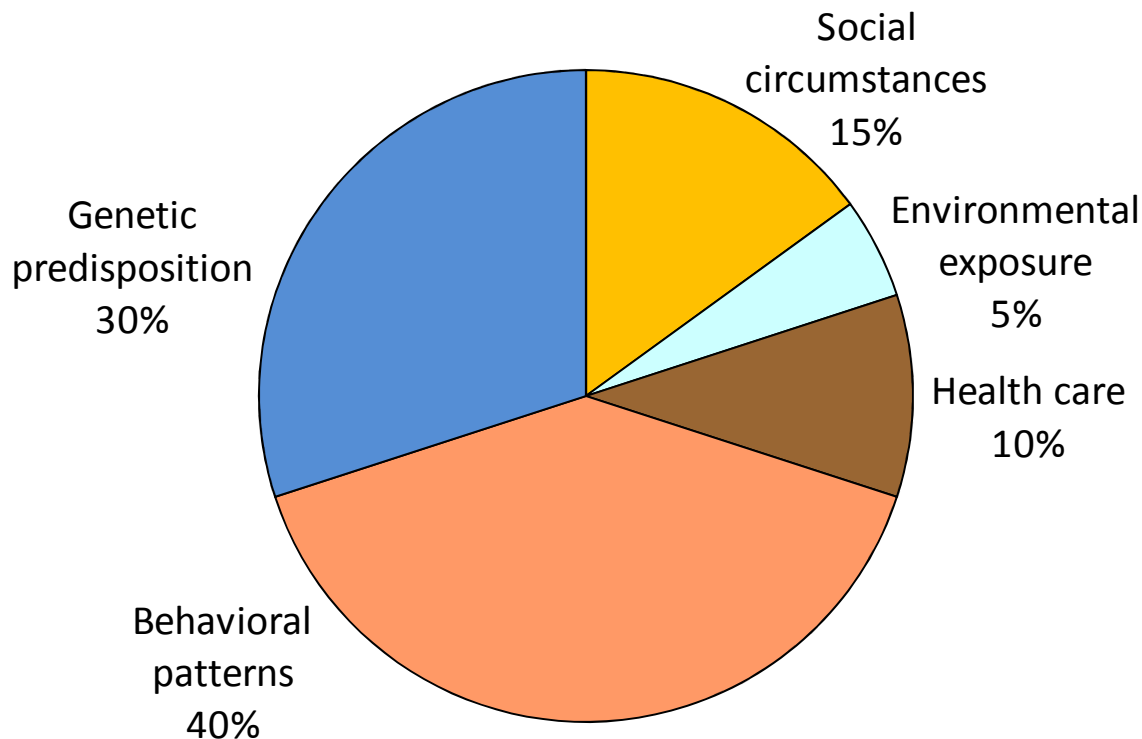
Data: OECD Health Data 2005 and 2006.

Drivers of a Low-Value Health System



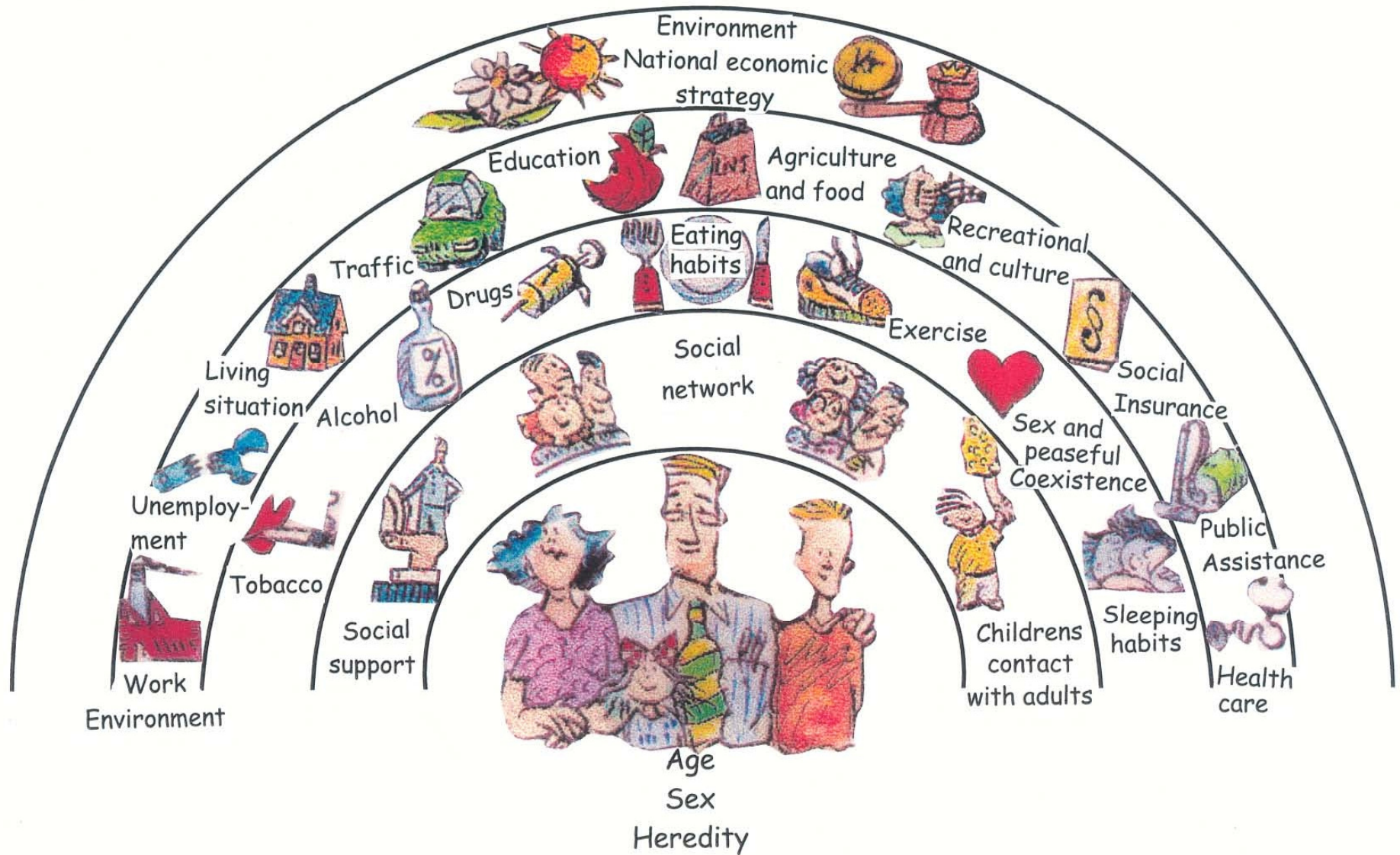
Determinants of Health and Their Contribution to Premature Death

Proportional Contribution to Premature Death



Adapted from: McGinnis JM, Williams-Russo P, Knickman JR. The case for more active policy attention to health promotion. *Health Aff (Millwood)* 2002;21(2):78-93.

Determinants of health





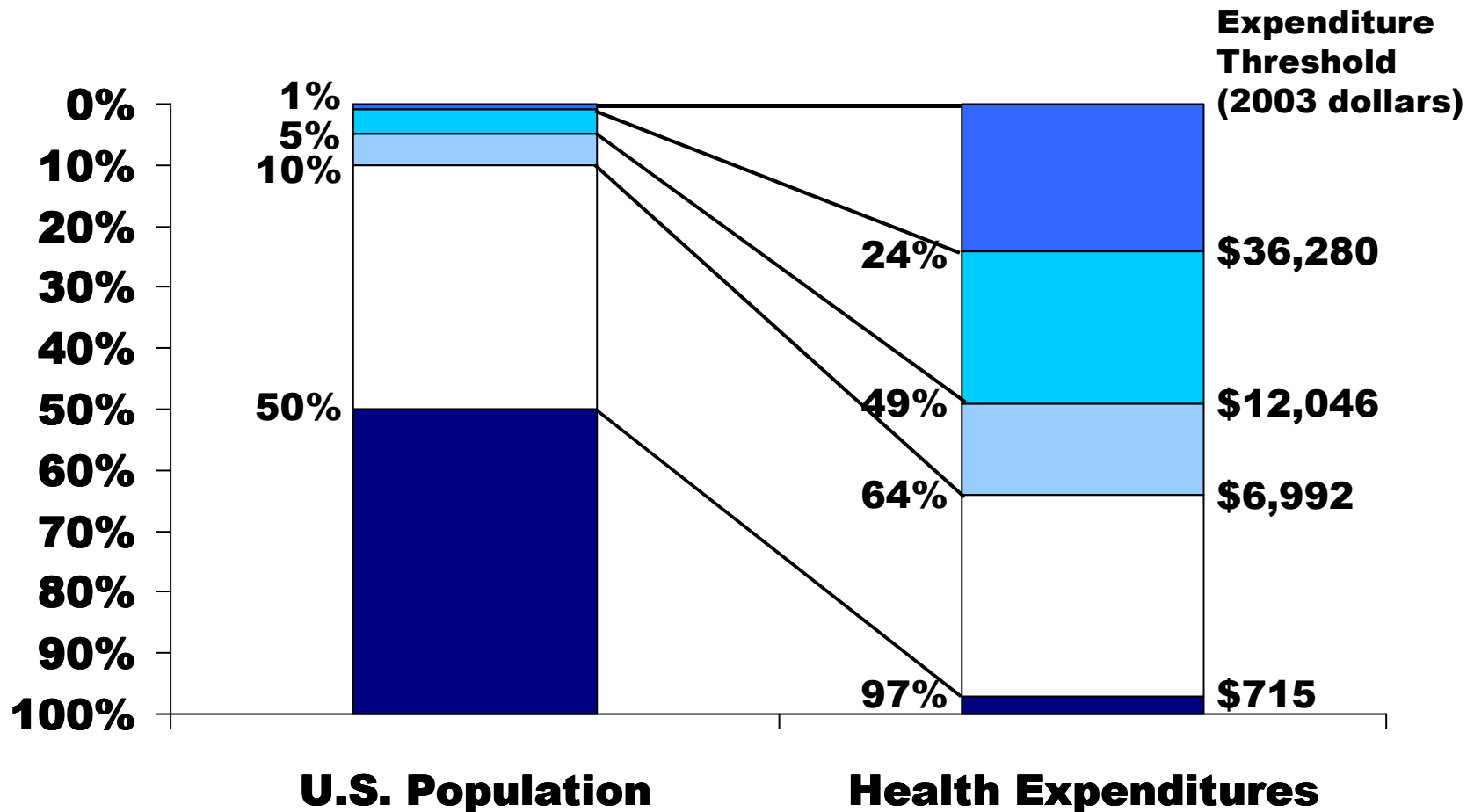
Overview of Triple Aim Initiative

Elements of a Triple Aim Enterprise

- Population
- Macro-integrator
- Strategy aligned with Triple Aim
- Focus on a set of design concepts

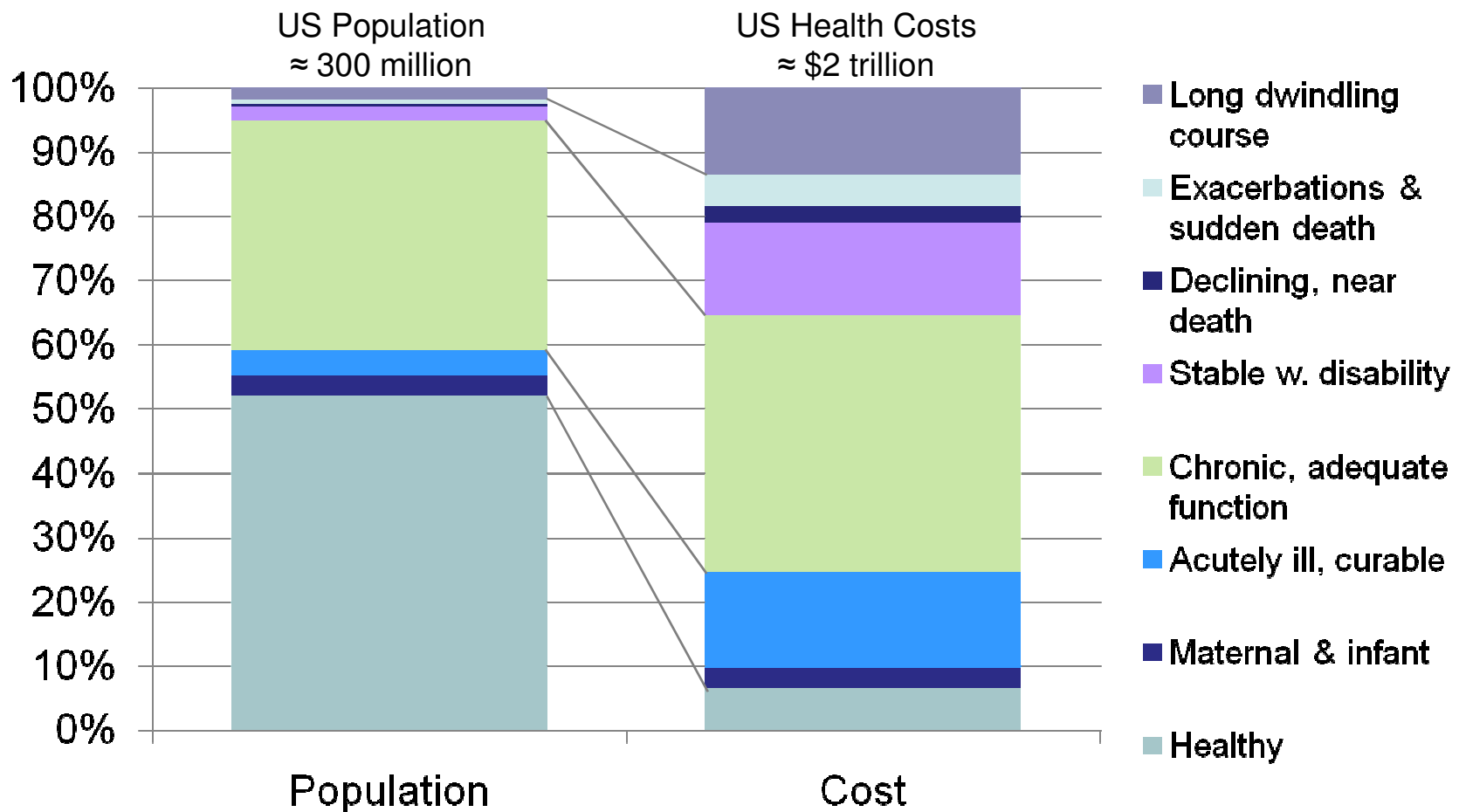
Health Care Costs Are Concentrated in Sick Few— The Sickest 10 % Account for 64 % of Expenses

Distribution of Health Expenditures for the U.S. Population, by Magnitude of Expenditure, 2003



Source: The Commonwealth Fund. Data from S. H. Zuvekas and J. W. Cohen, "Prescription Drugs and the Changing Concentration of Health Care Expenditures," *Health Affairs*, Jan./Feb. 2007 26(1):249–57.

Population Segmentation by Health Status



From Joanne Lynn et al, Bridges to Excellence, *Milbank Quarterly*, Vol. 85, No. 2, 2007

Bellin's Enterprise Population

- **Description and size:**
1000 employers throughout the region
- **Overall aims:**
To improve health, reduce costs and increase readiness and confidence levels for employers and individuals

GroupHealth Enterprise Population



- Description and size:
570,000 people, with 360,000 getting care within owned and operated clinics
- Overall aims:
90th %ile in HEDIS and CHAPS
10% below market costs
- Measures:
Process measures (defects measures)
HEDIS, CHAPS, specialty specific initiatives

Colorado Examples

- Colorado Access: ≈2500 disabled Medicaid-only Adults (21-64) in 6 metropolitan counties
- North Colorado Health Alliance: ≈5600 behavioral health and primary care patients with co-morbid mental health diagnoses
- Kaiser Permanente Colorado: 475,000 members

Selecting a Defined Population

- Population is known:
 - Individuals in the population can be enumerated with reasonable accuracy.
 - Per capita cost can be estimated with reasonable precision.
 - Some information about variation in overall health of individuals is available.
- The population is somewhat stable.
- Large enough to be meaningful
 - 10,000's to 100,000's
- Generally not focused on a specific disease or condition

Macro-Integrator

- It may or may not be a new structure or organization.
- It pulls together the resources to support a defined population.
- It builds alliances and coalitions.
- It optimizes the Triple Aim for the sake of a defined population.
- It works with and helps to improve micro-systems to support individuals.

Current Triple Aim Sites

- **Hospital-Based Systems**

- Cape Fear Valley (NC)
- Bellin Health (WI)
- Cincinnati Children's Hospital Medical Center (OH)
- Genesys Health (MI) (Ascension)
- ThedaCare (WI)

- **Health Plans**

- Blue Cross Blue Shield of Michigan (MI)
- CareOregon (OR)
- Colorado Access (CO)**
- Eastern Carolina Community Plan (NC)
- New York-Presbyterian System SelectHealth, LLC (NY)
- UPMC Health Plan (PA)
- Independent Health (NY)
- Wellmark (IA)

- **Integrated Health Systems**

- Group Health (WA)
- HealthPartners (MN)
- Kaiser Permanente, Colorado Region (CO)**
- Kaiser Permanente, Mid-Atlantic Region (MD)
- Martin's Point Health Care (ME)
- Presbyterian Healthcare (NM)
- Southcentral Foundation and Alaska Native Medical Center (AK)
- Veterans Health System:
 - VISN 10—Cincinnati VAMC (OH)
 - VISN 20—Portland VAMC (OR)
 - VISN 23—Nebraska, Western Iowa VAMC (NE)

- **Public Health Department**

- King County Department of Public Health (WA)

- **State Initiative**

- Vermont Blueprint for Health (VT)

- **Safety Net**

- Contra Costa Health Services (CA)*
- North Colorado Health Alliance (CO)**
- Primary Care Coalition Montgomery County (MD)
- Queens Health Network (NY)

- **Employers/Businesses**

- QuadGraphics/QuadMed (WI)

- **International**

- Blackburn With Darwen Primary Care Trust (England)
- Bolton Primary Care Trust (England)
- Central East Local Health Integration Network (Canada)
- East Lancashire Teaching Primary Care Trust (England)
- Eastern and Coastal Kent Primary Care Trust (England)
- Forth Valley (Scotland)
- Herefordshire Primary Care Trust (England)
- IMPACT BC (Canada)
- Jönköping (Sweden)
- Tayside (Scotland)

- **Social Services**

- Common Ground (NY)

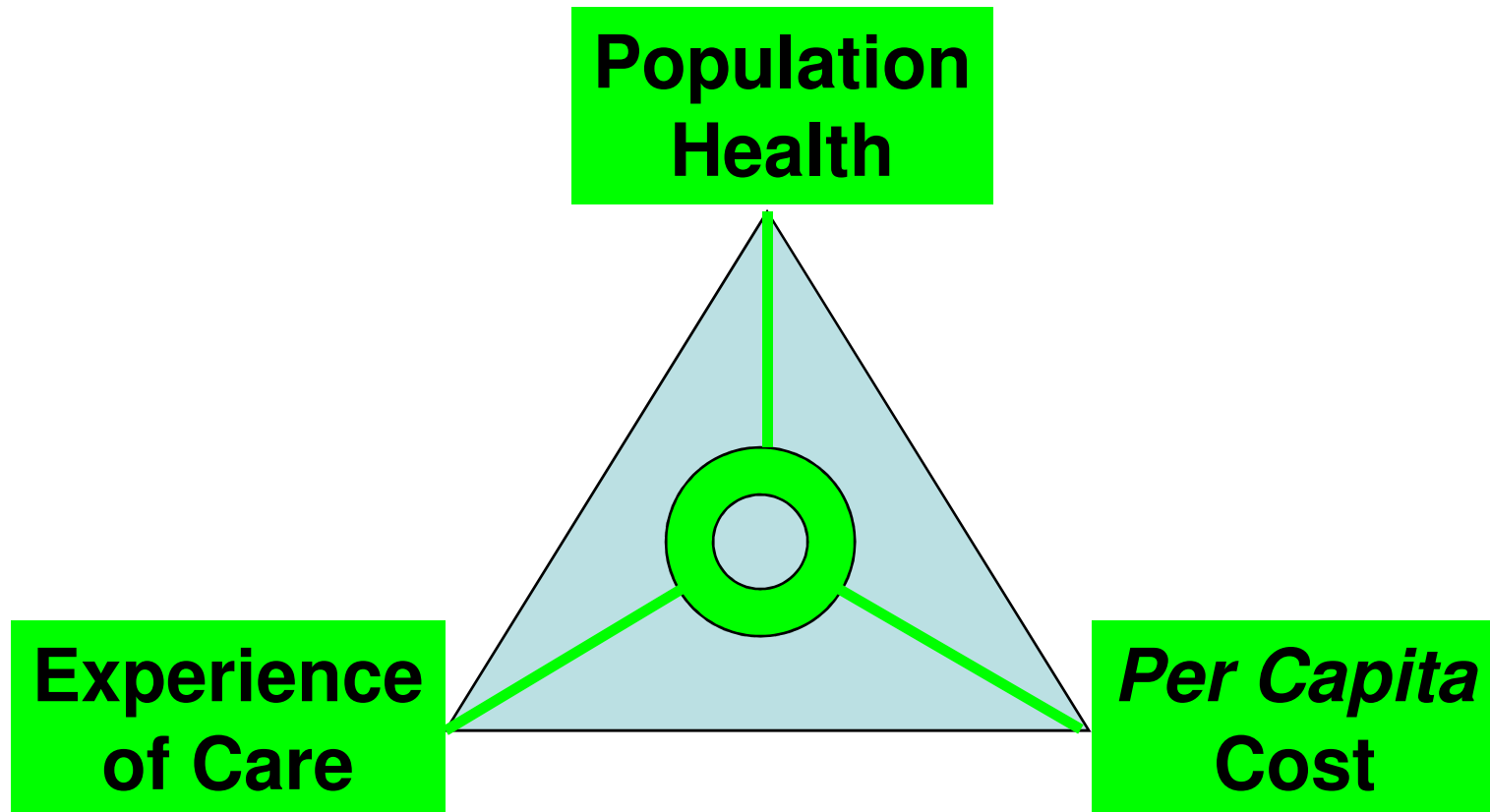


Updated: December 5, 2008

System of Improvement for the Triple Aim

- Level of ambition and consistency with organizational priorities
- Methods to form coalitions
- System level measures to define the aim for a given population
- Themes or segments for guiding system changes
- Prototype testing beginning with the individual and family
- Scale-up and spread
- Anchoring the new approach in the community

Three Dimensions of Value

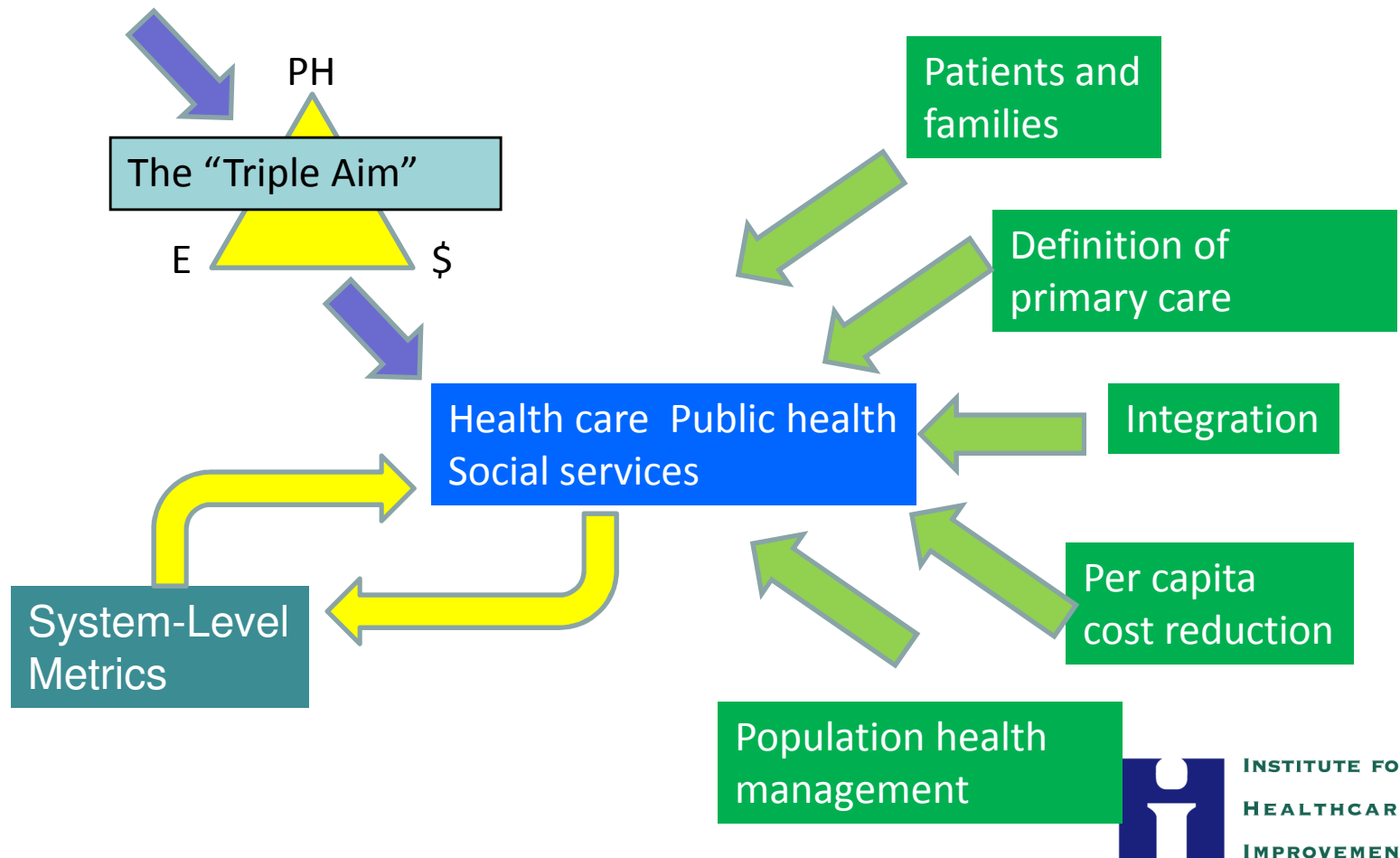




Triple Aim Design

Design of a Triple Aim Enterprise

Define “Quality” from the perspective of an individual member of a defined population



Potential Triple Aim Outcome Measures

Dimension	Measure
Population Health	1. Health adjusted life expectancy (HALE) (life expectancy and self-rated health status)
	2. Composite Health Risk Appraisal score
	3. Hospital and ED utilization for ambulatory care sensitive conditions
	4. Disease burden
Per Capita Cost	1. Cost per member of the population per month
	2. Hospital and ED utilization costs
Patient Experience of Care	1. Standard question focused on a patient's overall experience
	2. Key question(s) from current patient survey

Health Adjusted Life Expectancy (HALE)

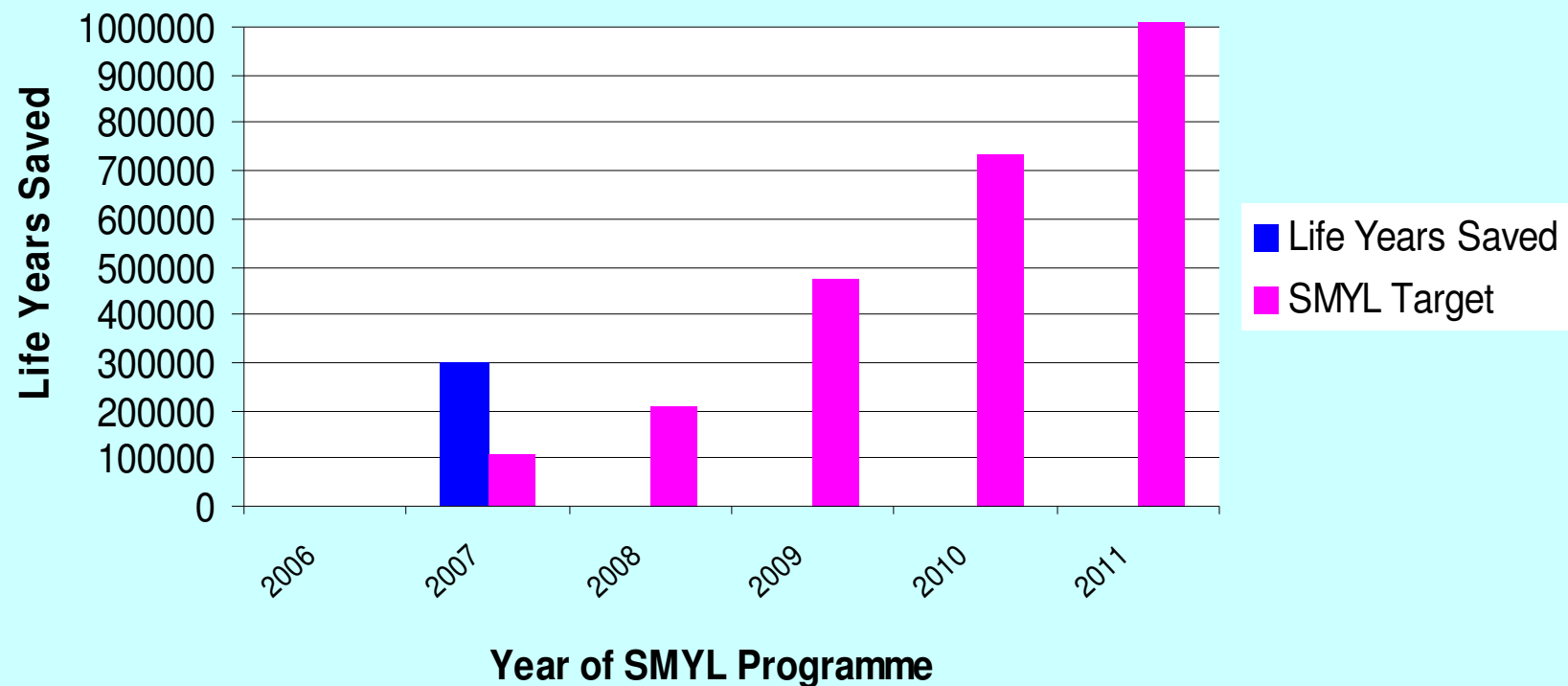
HALE = Product of average life expectancy and % rating their health status good or higher

- Average Life Expectancy = Average number of years a baby born in a particular year is expected to live if current age-specific mortality trends continue to apply
- Self-rated Health Status = Response to the question “How is your general health?” on a 5 pt. scale from poor to excellent

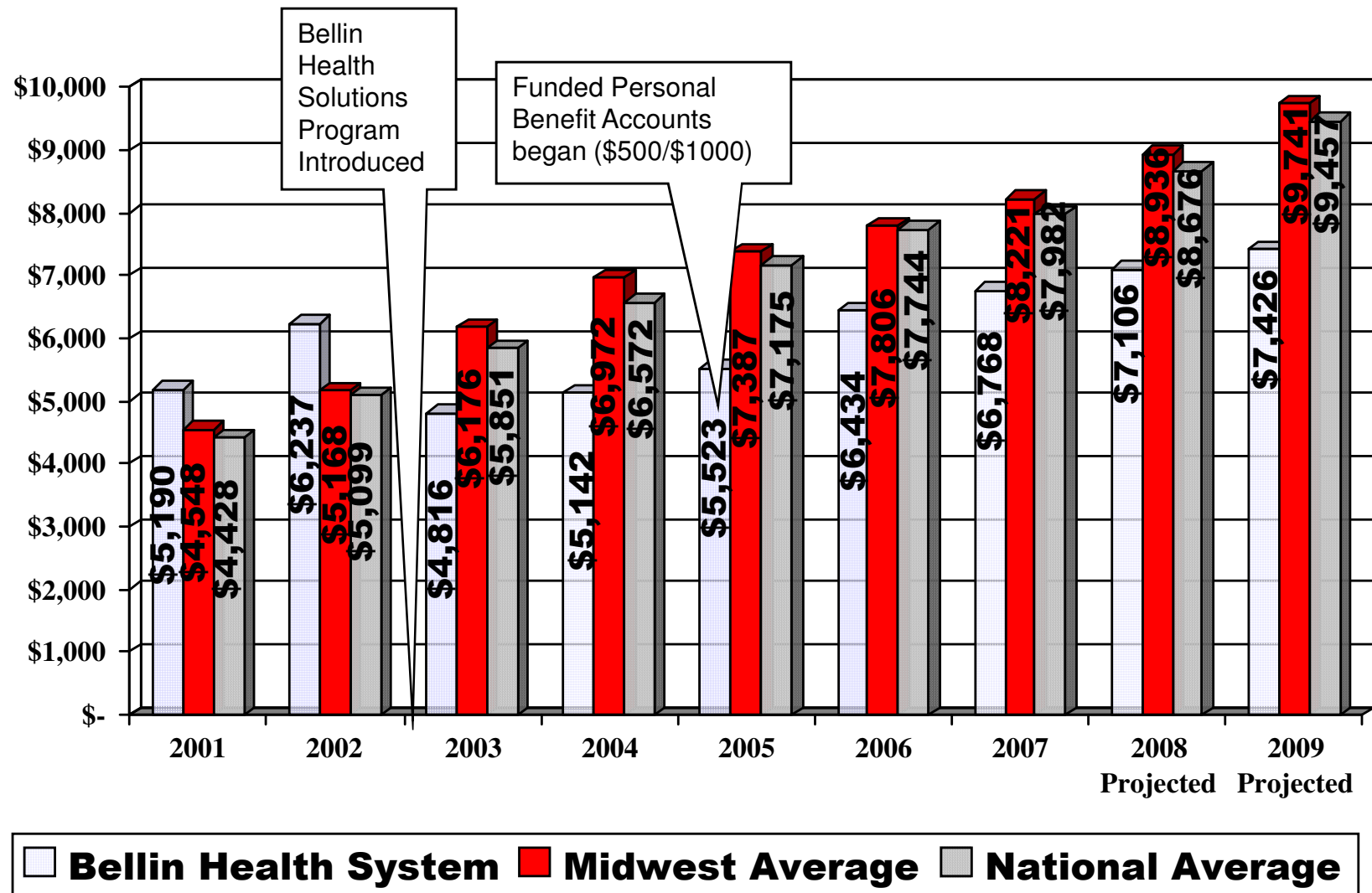
Kindig, D. *Purchasing Population Health*, 1997

Save a Million Years of Life (SMYL)- Reducing the Life Expectancy Gap in East Lancashire

Life Years Saved - 2007-2011



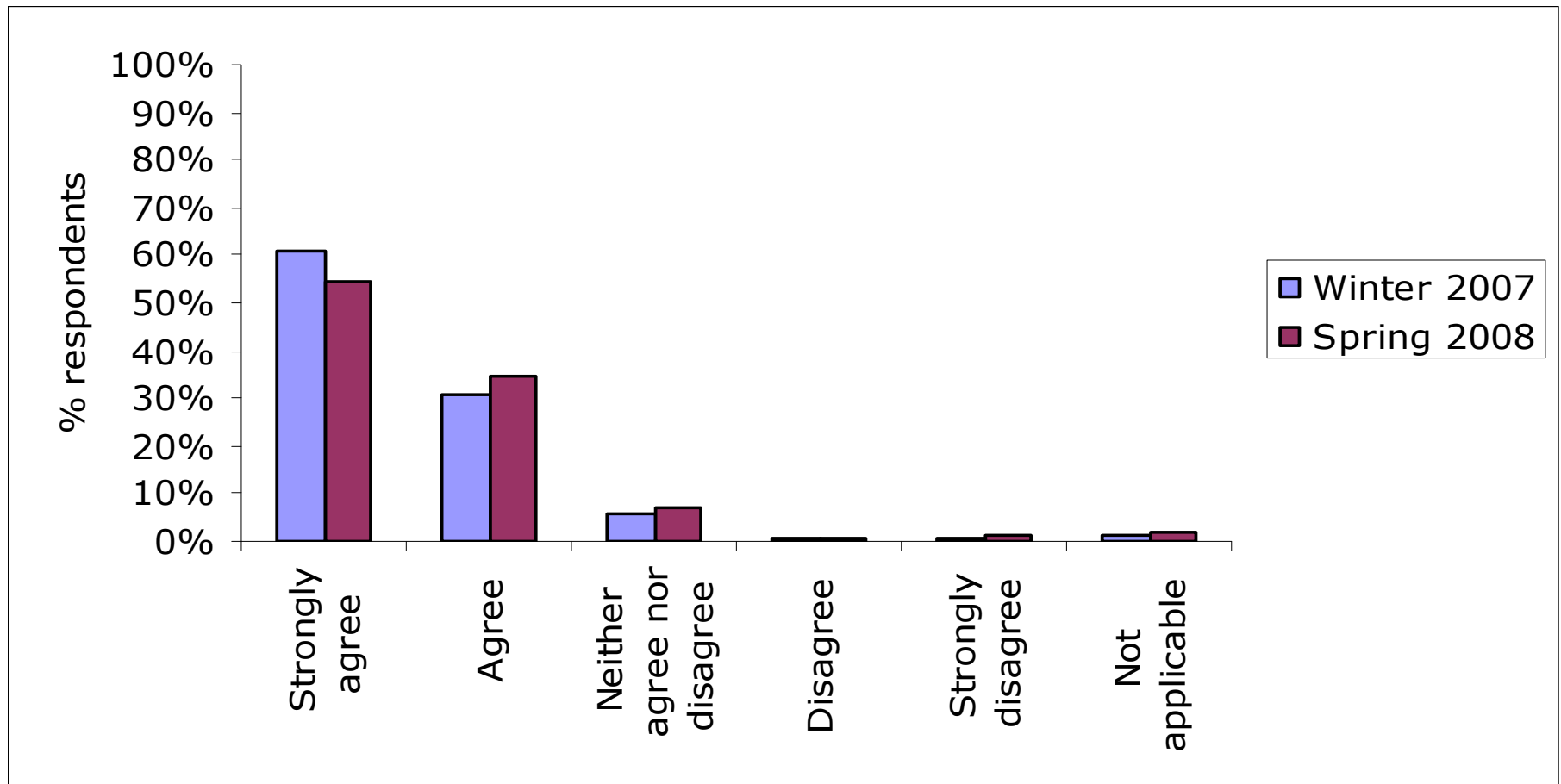
Bellin Health – Cost Employee Plan vs. Averages



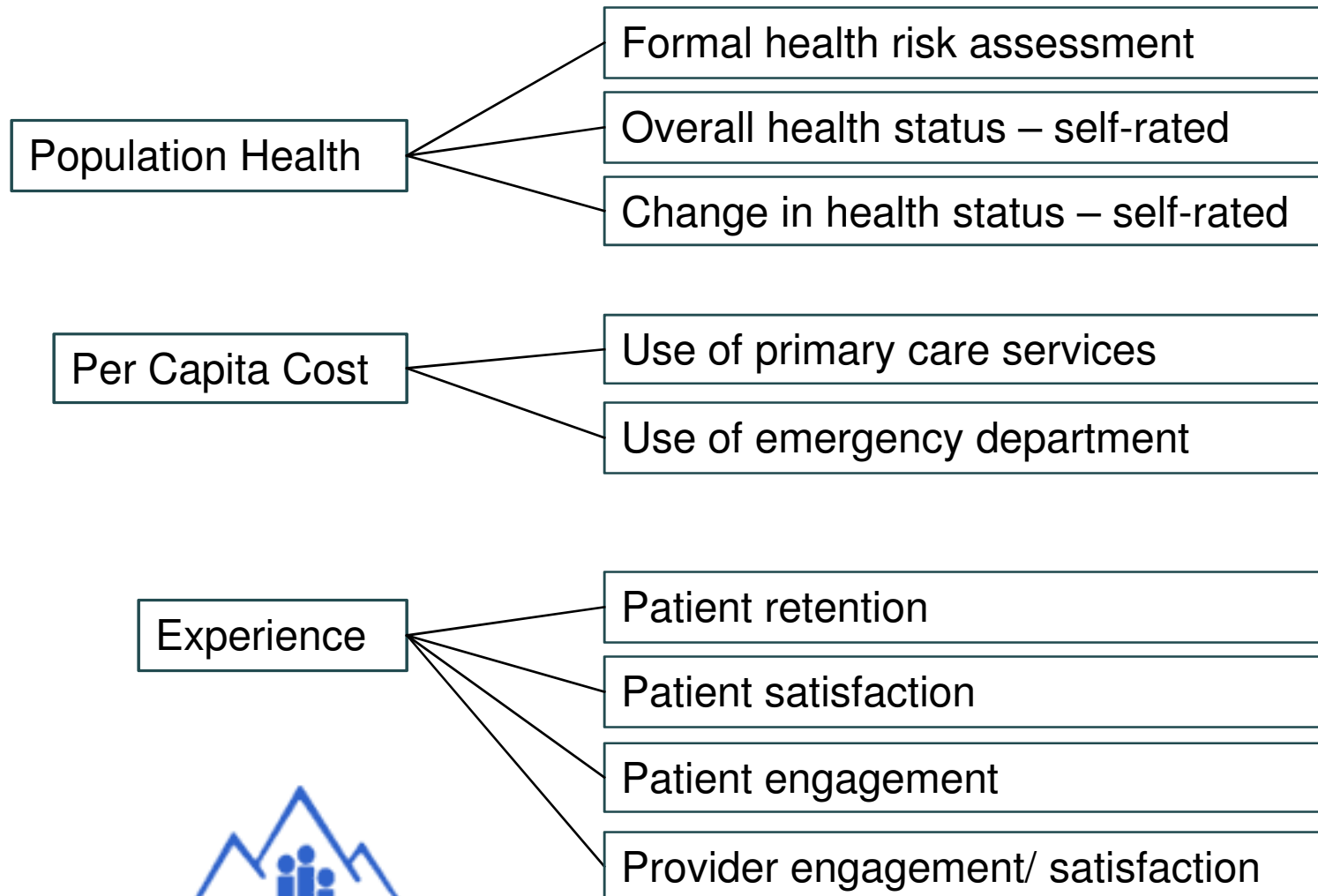
Patient Experience of Care (PCR Clinics)

Care Oregon

Question: I would recommend my care team to family and friends.



Access Health Plan Metrics



Triple Aim Design Components

1. Individuals and families
2. Redesign of “primary care” services and structures
3. Population health management
4. Cost control platform
5. System integration

1. Individuals and Families

- For medically and socially complex patients, establish partnerships among individuals, families and caregivers, including identifying a family member or friend who will be supported and developed to coordinate services among multiple providers of care.
- Jointly plan and customize care at the level of the individual.
- Actively learn from the patient and family to inform work for the population.
- Enable individuals and families to better manage their own health.



Results – Experience

GENESYS
HEALTHWORKS



Physician: "We have discussed your medications, now let's talk about your health goals."

Patient: "I really want to lose weight."

Physician: "Great, that is very important to help manage your blood pressure. Our Health Navigator will follow up with you to help you make a plan for healthier diet and exercise."



Health Navigator: "Since you're in the grocery store, do you want me to call you back later?"

Patient: "No, This is perfect timing. Maybe you can help me make some better food choices while I am shopping!"

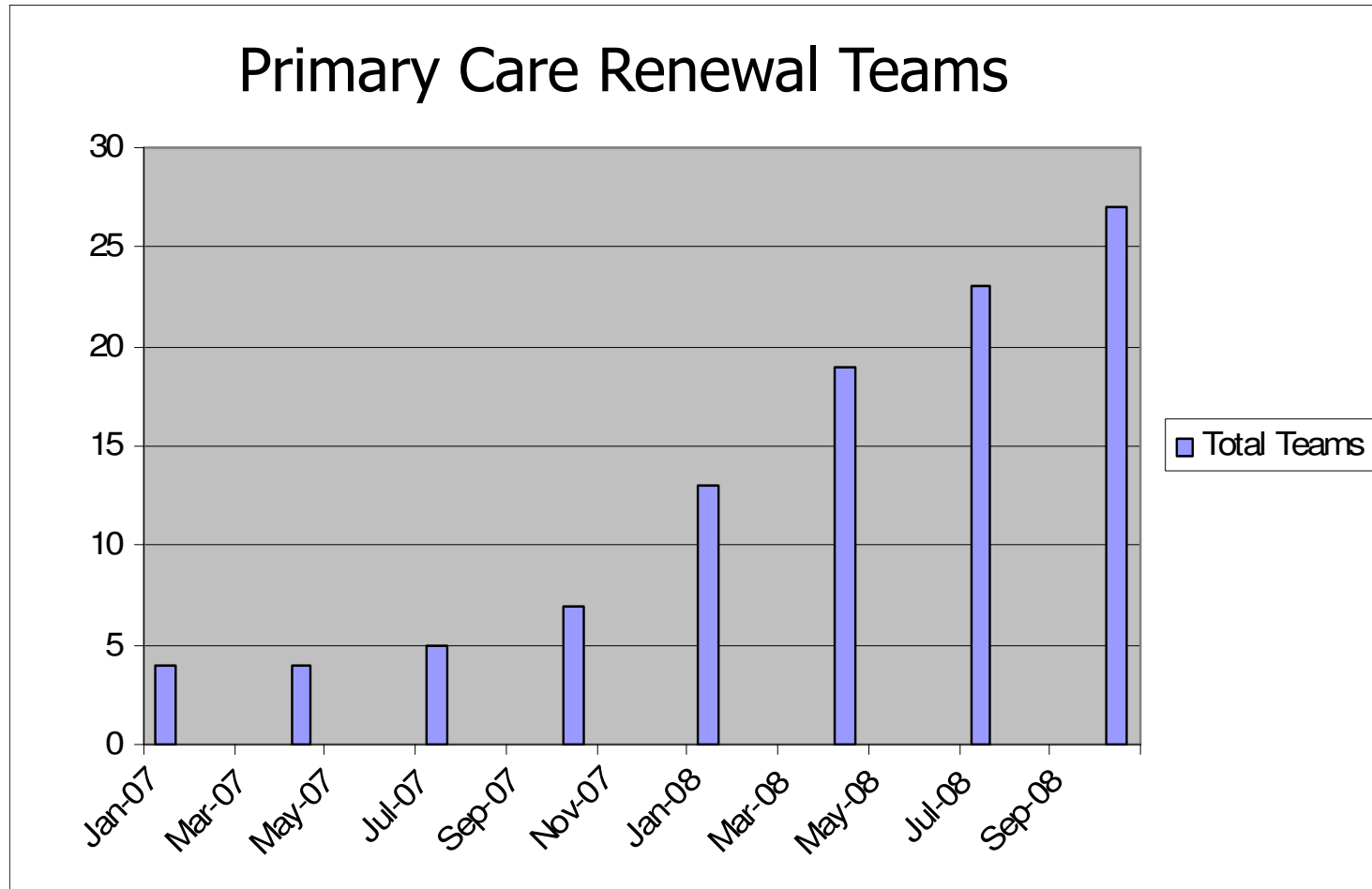
2. Redesign of “Primary Care” Services and Structures

- Have a team for basic services that can deliver at least 70% of the necessary medical and health-related social services to the population.
- Deliberately build an access platform for maximum flexibility to provide customized health care for the needs of patients, families, and providers.
- Cooperate and coordinate with other specialties, hospitals, and community services related to health.

Care Oregon's Work

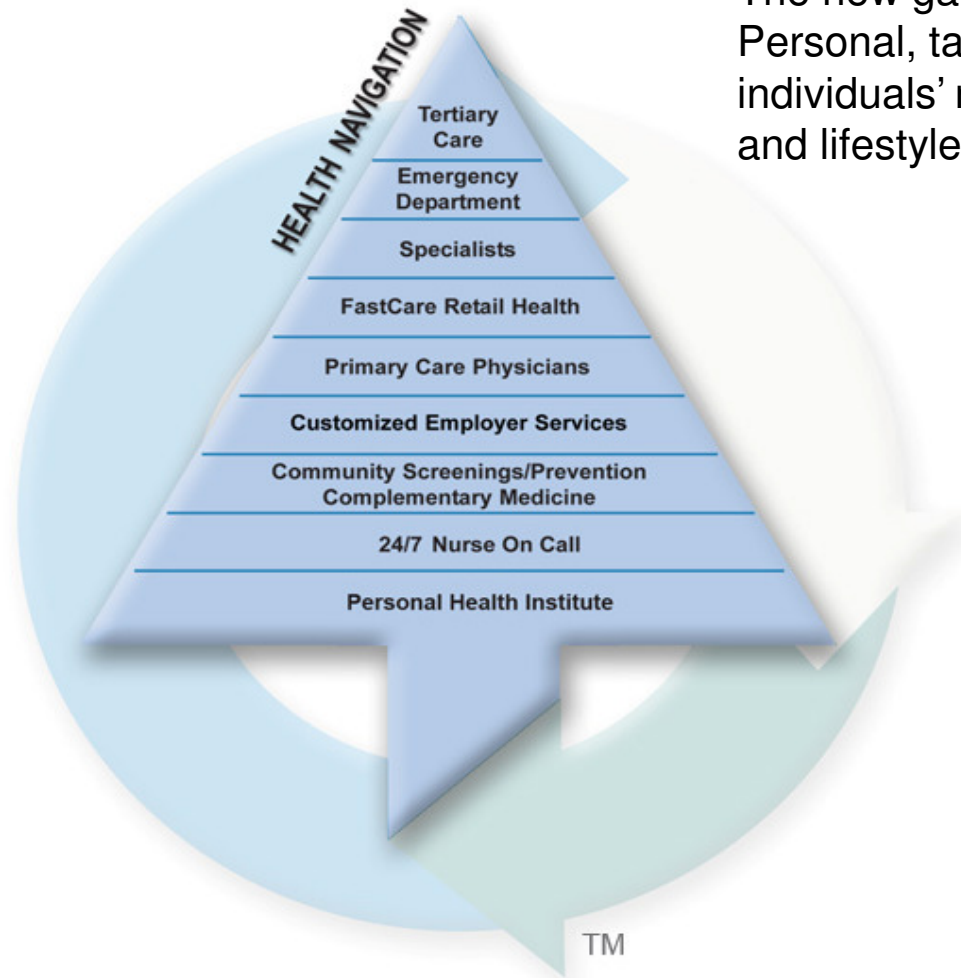
- Primary care renewal:
 - Model design
 - Team based care
 - Proactive panel health improvement
 - Integrated behavioral health
 - Customer driven, evidence based care
 - Barrier free access
 - Continuous improvement infrastructure
 - Collaborative co design and learning network
 - Pilot payment methodology

Spread for Care Oregon



Bellin Health Navigation

The new gateway to Bellin Health. Personal, tailored treatment to individuals' needs, learning styles and lifestyles.



bellinhealth

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**INSTITUTE FOR
HEALTHCARE
IMPROVEMENT**

Health Partners' Medical Home Results

Health Outcomes	Consumer Experience	Affordability
Staying Healthy - Breast Cancer Screening¹ - Cervical cancer Screening¹ - Colorectal Cancer Screening¹ - Cancer Cancer Combined¹ - Chlamydia Screening in Women¹ - Childhood Immunizations¹ <i>Health Assessment Completion %²</i> - Optimal Lifestyle² - Preventive Services Up to Date²	Satisfaction - Access³ - Communication³ - Coordination³ - Quality of Care³ - Decision Making³	Total cost of care Total, Diabetes, Heart Failure, Other Chronic Conditions²
	Avoidable Events - Unplanned readmissions to the hospital (# of Occurrences)² - Preventable/unnecessary ER visits (# of Occurrences)²	Appropriate use of hospital resources - Rate of PAD (potentially avoidable delays)² - Rate of NQA (non-qualified admissions)² - Length of Stay: All, Diabetes, CHF, Other Chronic Conditions² - Admit Rate: All, Diabetes, CHF, Other Chronic Conditions²
Getting Better - Treatment for Children with URI¹ - Testing Children for Pharyngitis¹	Coordination of Care % of Patients seeing 10 or more physicians²	Avoidable Events - Unplanned readmissions to the hospital (value)² - Preventable/unnecessary ER visits (value)²
Living with Illness - Controlling High Blood Pressure¹ - Optimal Diabetes Care¹ - Optimal Vascular Care¹ - Use of Appropriate Medications for People with Asthma¹ <i>CHF - LV Assessment; ACE/ARB Use²</i> <i>Depression - PHQ9 Use, Remission Rate²</i> - Optimal Depression Care²	End-of-Life Care - Advanced Directives² - Hospice Referral Timing² - ICU in the last 14 days of life²	Pharmacy Services Generic utilization rate²

Sources:

- Minnesota Community Measurement
- HealthPartners Measurement
- HealthPartners Consumer Choice Survey

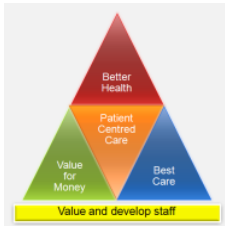
Gray Italics: denotes future population health measures

Phases of Medical Home

- **Level One: Caring for a defined population or list**
 - Defined list – patient panel, registered list – and responsibility for the list of patients;
 - Ability to generate disease registries (ideally computerized); ability to track requirements for effective intervention;
 - Provision for ‘ad hoc’ contacts – e.g. after hours phone access, urgent-care visits (walk-in center), email?
- **Level Two: Delivering team-based care**
 - Care delivered by a team – not all doctors; all working at the top of their license;
 - Same day access – delays in access will divert to other care locations.
- **Level Three: Redefining relationship to specialty care**
 - Redefinition of role of specialists relative to primary care;
 - Movement of care from just illness care to include secondary prevention (optimal management of already existing health issues).
- **Level Four: Shifting to delivering “health” rather than “disease care”**
 - Effective incorporation of primary prevention, including connectivity to other community resources.

3. Population Health Management

- Efficiently customize services based on appropriate segmentation of the population.
- Use predictive models and health risk assessments that take into account situational factors, medical history, and prior resource utilization to deploy resources to high-risk individuals.
- Work with the community to advocate and provide incentives for smoking prevention, healthy eating, exercise, and reduction of substance abuse. Provide assistance for all who wish to make health-related behavior change.
- Set and execute strategic initiatives related to reducing inequitable variation in outcomes or undesirable variation in clinical practice.



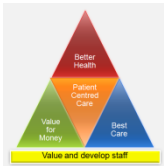
What Are We Trying To Accomplish ?

Aims:

- Reduce health inequalities for cardio-vascular disease and improve life expectancy for all residents in Bolton aged 45+.
- Understand A&E utilisation and preferred care access models.
- Work in partnership between public health, local authority and primary care to improve health experience and wellbeing.

Outcomes:

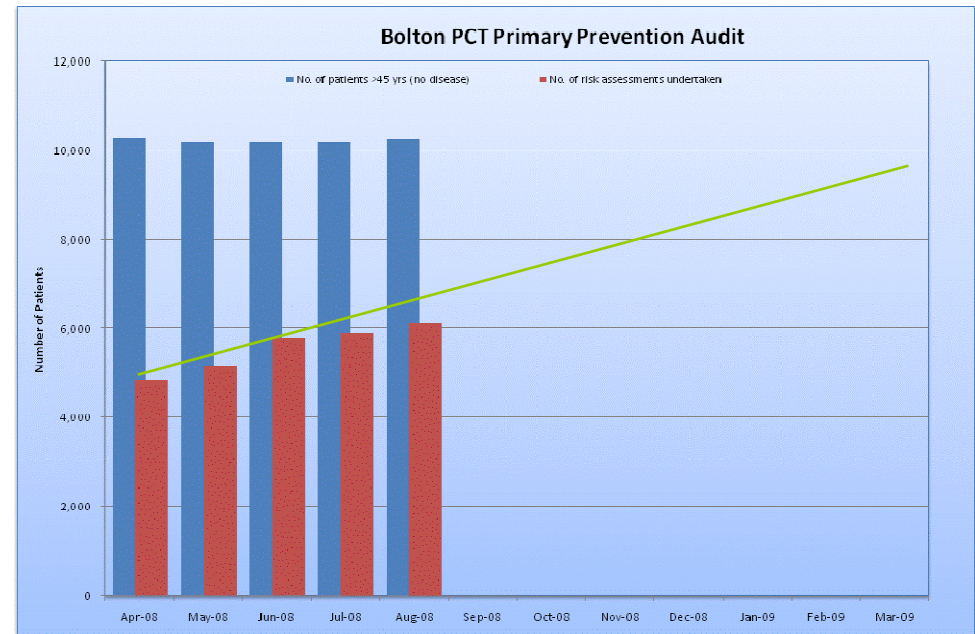
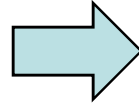
- Risk assess 100% of all residents aged 45+ for CVD by April 2009.
- Smoking cessation activity increased
- For patients with risk rating of >20% apply primary care prevention strategies.
- Improve utilisation of A&E and development of unscheduled care facilities.



CARDIOVASCULAR SCREENING PROGRAMME PROGRESS

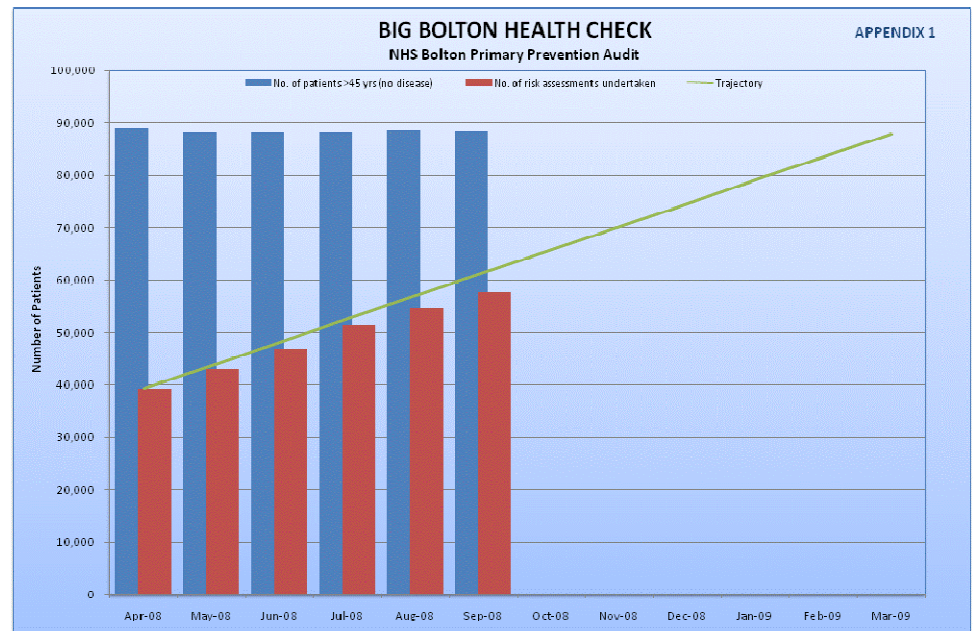
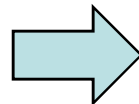
Farnworth:

- 59% screened (6121pts)
- To test 6 health trainers to target open sessions
- Priority to target businesses
- Social marketing:

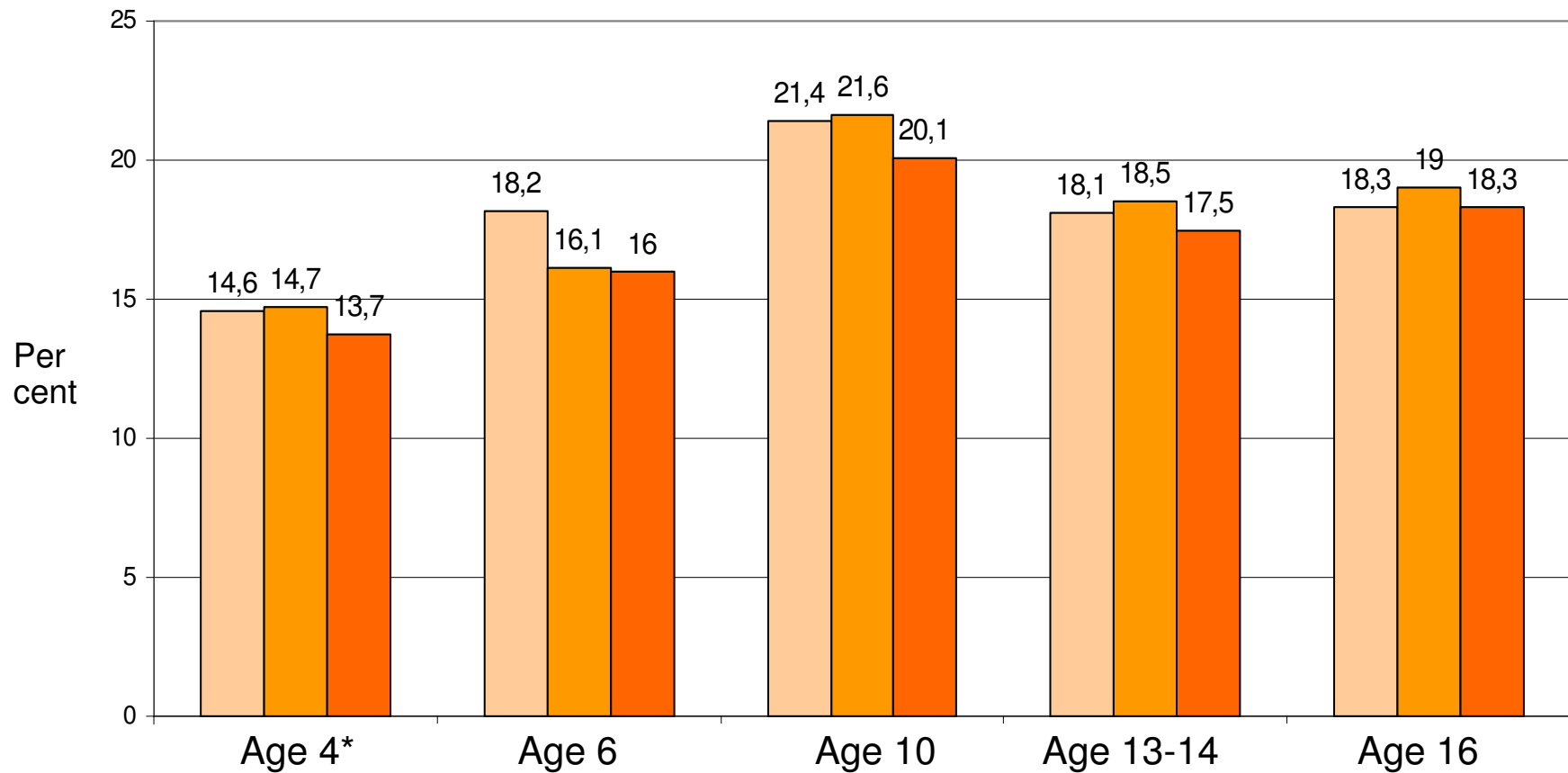


Early expansion to all Bolton:

- 65% screened (57,000 pts)
- Priority to expand business links
- Target lists
- Social marketing



Part of children with obesity and obese during the school years of 2004-05, 2005-06, 2006-07, Jönköping County



The first measure is the NOW-situation

*Age 4 = 2004/2005 = born year 2000
 2005/2006 = born year 2001
 2006/2007 = born year 2002

2004/2005 2005/2006 2006/2007

4. Cost Control Platform

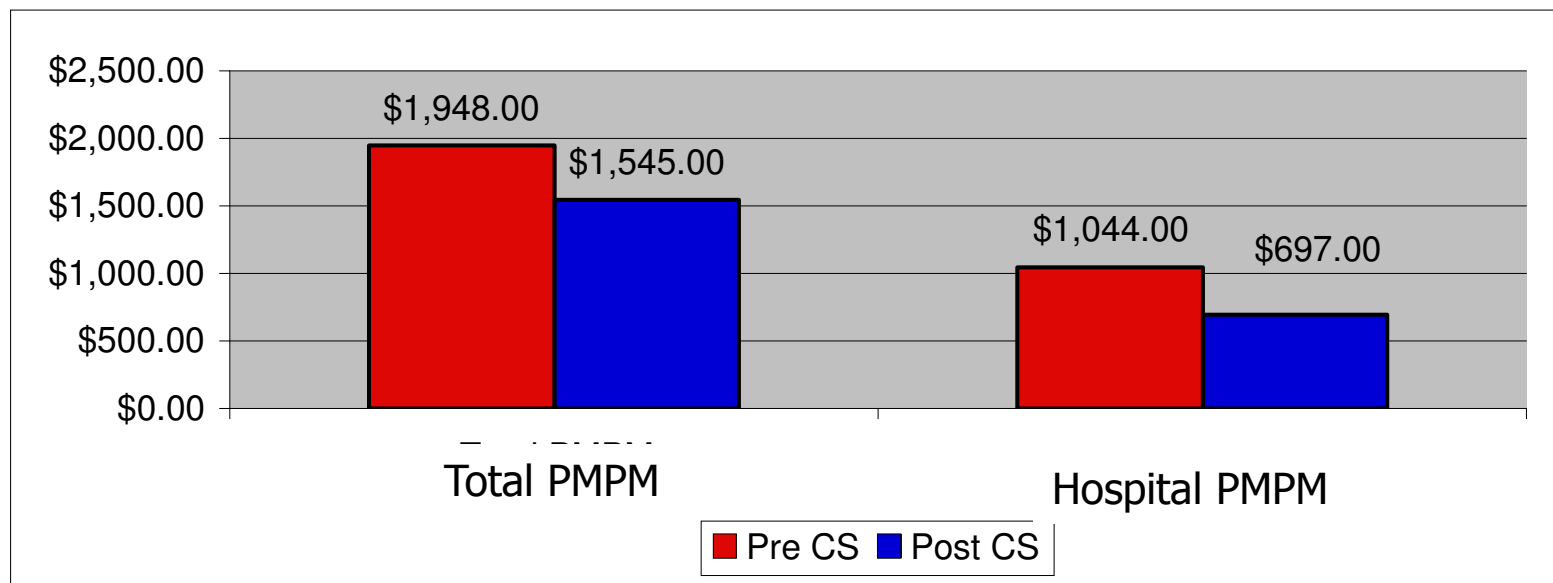
- Achieve $< 3\%$ inflation yearly for per capita cost by developing cooperative relationships with physician groups and other health care organizations committed to reducing the waste of health care resources.
- Achieve lowest decile performance in the Dartmouth Atlas measures by breaking or countering incentives for supply-driven care.
- Reward health care providers, hospitals, and health care systems for their contribution to producing better health for the population and not just producing more health care.
- Orient care over time—the “patient journey”—targeted to the best feasible outcomes.

Care Oregon

- CareSupport:
 - Motivational interviewing approach for chronic disease behavior change
 - Transitional care outreach to hospitalized members
 - Building integrated partnerships with social service agencies
 - Broader and deeper integration with primary care network
 - Individual level learning for the population (case studies)

Care Oregon Cost per Capita: PMPM Total for CareSupport Enrolled Members

Cost for CareSupport Enrolled Members (N=1991) during the time period of Jan. 1, 2007 and May 31, 2008. Pre-CS = claims paid for 12 months prior to enrollment in CareSupport and Post-CS = claims paid for 12 months following enrollment in CareSupport.

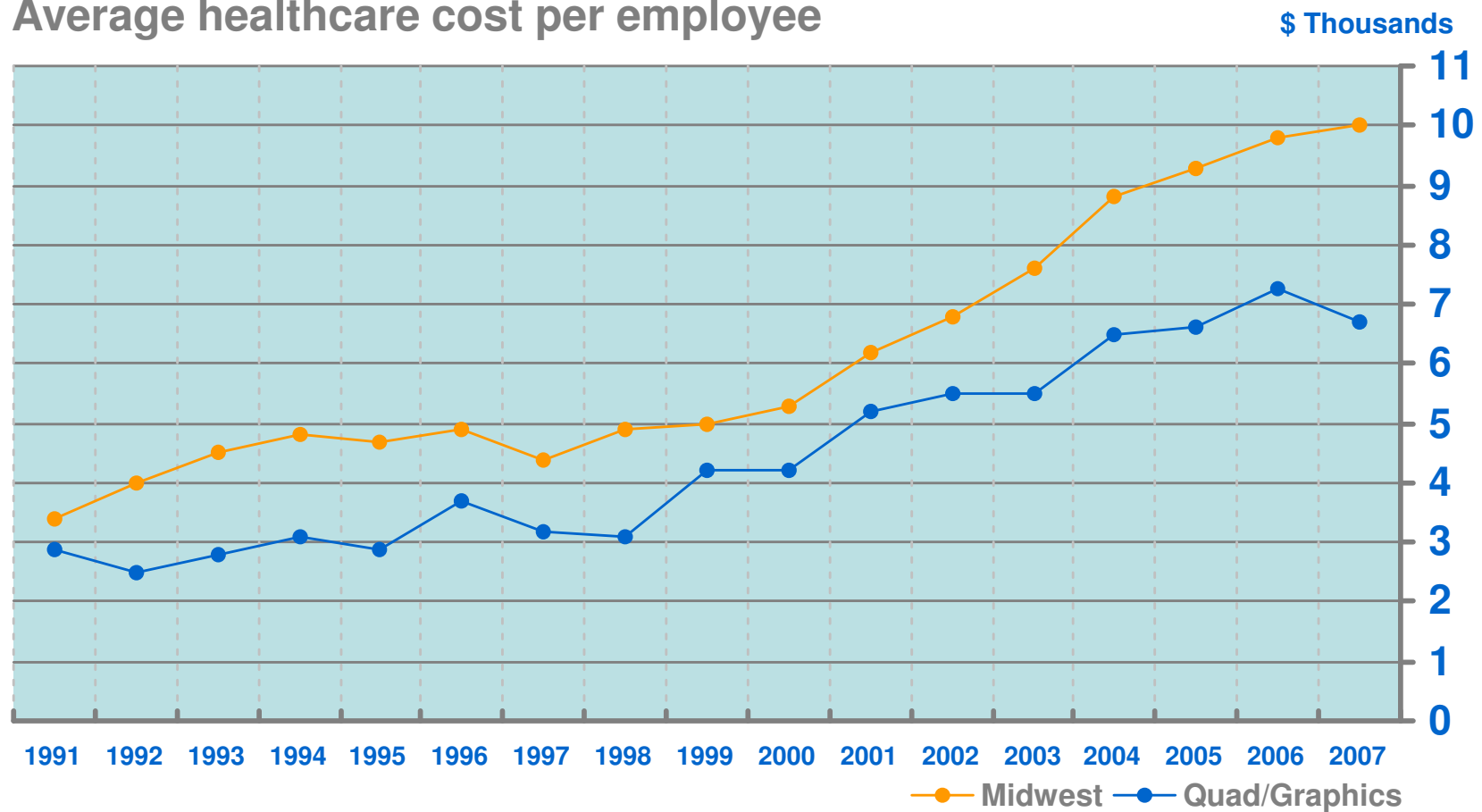


QuadMed Strategy

- **Provide** on-site primary care & selected specialty care
- **Focus** on prevention and wellness
- **Restructure** the delivery of primary care
 - Salaried providers, not “Production” based reimbursement
 - Incentives based on quality – Customer Satisfaction, adherence to guidelines, preventive services, collegiality, committee participation
 - Provide ample “face time” with patients
- **Provide** specialty care & hospital care through direct contracting - “Narrow Networks”
- **Remove** waste and improve Quality
- **Integrate** workers compensation into primary care services

QuadMed Track Record

Average healthcare cost per employee



Mercer

Mercer Consulting Study

Quad/Graphics' healthcare costs are consistently below the benchmark, when adjusted for demographics and benefit design:

- **18% below in 1998**
- **19% below in 2000**
- **17% below in 2002**
- **26% below in 2004**
- **32% below in 2006**



5. System Integration

- Match capacity and demand for health care and social services across suppliers.
- Insure that strategic planning and execution with all suppliers including hospitals and physician practices are informed by the needs of the population.
- Develop a system for ongoing learning and improvement.
- Institute a sustainable governance and financial structure for the Triple Aim system.



North Colorado Health Alliance Community Partnership

Banner Health – North Colorado Medical Center & North Colorado Family Medicine

Island Grove Regional Treatment Center, Inc

North Range Behavioral Health

Sunrise Community Health

Weld County Department of Public Health & Environment

Weld County Medical Society

North Colorado Medical Center Foundation

NCMC, Inc

United Way of Weld County

University of Northern Colorado – College of Natural & Health Sciences

Weld County Department of Human Services

Weld County Government – Board of County Commissioners

Mission

“The North Colorado Health Alliance will ensure that all underserved residents of North Colorado have access to appropriate, affordable, comprehensive, quality health care.”

Slide 49

m1

NCHA partnership broad-based - either those who provide direct services within the safety net or those who support the agencies providing those services

mmoran 5/14/2008

NCHA partnership broad-based - either those who provide direct services within the safety net or those who support the agencies providing those services

MB: Partnership is evolving over time via: 1) mergers or formal affiliations, e.g., MCC and SCH, NRBH/IGRTC 2) changes in focus of the Alliance, e.g., integrated behavioral health services; 3) the definition of a Health Care Home expands and evolves

mmoran, 5/14/2008



Strategic Focus

Evidence – Based Service Development

**Primary Medical, Dental
Behavioral Health
Public Health**

**Access
to Care**

**Integrated
Services**

**Disease
Management**

**Outreach
&
Mobile Resources**

**Workforce
Development**

Medical/Dental Providers

**Behavioral Health
Providers**

**Allied
Health
Staff**

**Cultural
Competency**

**Integrating
Infrastructure**

Integrated HIT

**Centralized
Services**

**Outstation
Eligibility Staff**

**Community
Connection Center**

**Sustainable
Financing**

Advocacy

INTEGRATED HEALTH DELIVERY SYSTEM

Slide 50

m2

Focus on 3 pillars:

1 - expansion of access

2 - workforce development

3 - integrated infrastructure

mmoran, 5/11/2008



Expanding Access . . . Improving Quality . . . Eliminating Disparities

Mobile Medical Van



School Based Health Center



60,000 sf primary care clinic



SHARED Electronic Patient Record



Hospitalist



Prenatal



OB

Pediatrics



Specialty Care



ADHD



Public Health



Family Medicine Training



Slide 51

m3

YOU HAVE TO SEE THIS SLIDE WORK

on this slide and the next, describe NCHA projects / successes

give sense of scope of focus

didn't highlight Integrated Behavioral Health here - have slides dedicated to that topic later in presentation

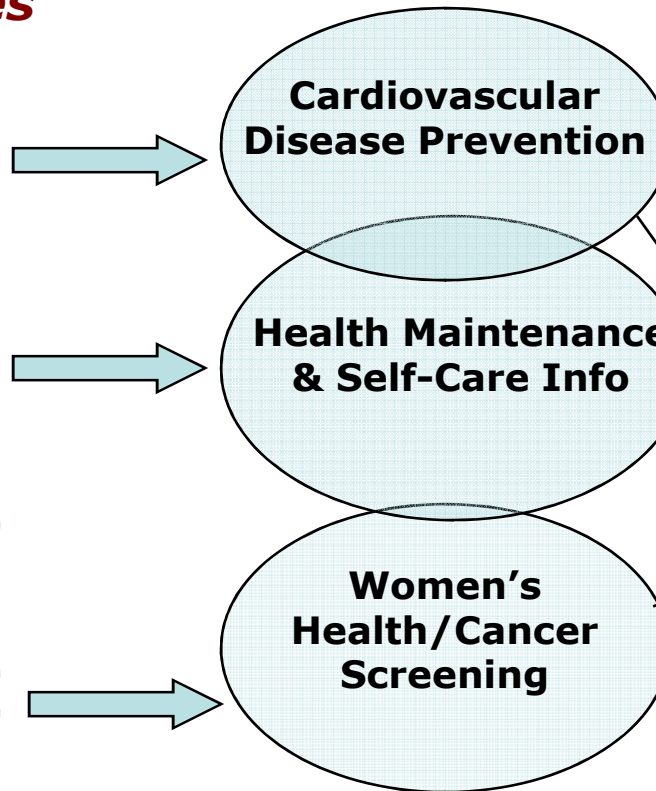
mmoran, 5/11/2008

Clinical Prevention and the Triple Aim

Prevention services

- [HTN & Lipid Control](#)
- [CV Risk Assessment](#)
- [Diabetes Prevention](#)
- [Tobacco Cessation](#)
- [Immunizations](#)
- [Flu Clinic](#)
- [Pre-Visit Prep](#)
- [Kp.org and Distance Learning Technologies](#)
- [Handouts, Letters & AVS](#)
- [“Healthwise” Programs](#)
- [Breast Cancer](#)
- [Cervical Cancer](#)
- [Colorectal Cancer](#)
- [Chlamydia](#)
- [Osteoporosis](#)
- [Gestational Diabetes](#)

Systems, Out-reach, Decision Support & Performance Data



Member Experience

Population Health Index
\$PMPM

Population Health

Cost per Capita

Dialogue, Coaching & Education

Programs to engage members

- [Dietary Counseling](#)
- [HEAL Initiatives](#)
- [Member Advisory Roles](#)
- [Workshops and Consults on Motivating Change](#)
- [Healthy Living Classes and Webinars](#)
- [Self-Management Classes and Webinars](#)
- [Health Goals & Action Plans](#)

Nutrition Services, Healthy Living and Self-Management Support

Version 9, March 4, 2009

Next Steps for Triple Aim

- Parallel cohorts: North America; Europe
- New prototyping initiatives:
 - Reducing Avoidable Emergency Department Use
 - Reducing Overuse in Specialty Care

Synergies

- State and Regional initiatives: Vermont; Colorado, Minnesota, Oregon, Genesee County, MI, Washington DC.
- Medical Home initiatives
- Accountable Care Organization prototyping
- NQF National Priorities Partnership, e.g. waste reduction
- IRS focus on hospital community benefits
- Other?

Three Dimensions of Value

